INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Afatinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00221
Version 5

Daily oral treatment without a rest week

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Afatinib blocks abnormal EGFR-family growth signals in selected lung cancers. Treatment aims to control advanced cancer and delay progression.

EGFR-FAMILY TARGETED THERAPY

Afatinib

Blocks abnormal growth signals

Activating EGFR mutation identified → Growth signalling blocked → Cancer growth slowed

Afatinib is an irreversible EGFR-family tyrosine kinase inhibitor. It interrupts signals involved in cancer-cell growth; the tumour mutation test and treatment setting determine whether it is suitable.

Report diarrhoea, rash, breathing changes and eye symptoms promptly.

Your treatment aim

Which setting?

NCCP 00221a covers adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

NCCP 00221 v5 · official regimen · EMA Giotrif product information (updated 11 May 2026)

Duration

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

What studies tell us

In studies of selected EGFR-mutated advanced lung cancers, afatinib delayed cancer progression compared with cisplatin/pemetrexed. The overall survival comparison in that trial was not statistically significant. A separate comparison with gefitinib also did not show a statistically significant overall-survival difference. Ask how the study population and alternatives match your situation.

Your own decision

Ask how the study population matches your situation and discuss alternatives, harms and remaining uncertainty.

LUX-Lung 3 primary PFS: Sequist et al., JCO 2013 · LUX-Lung 3/6 mature OS: Yang et al., Lancet Oncology 2015 · LUX-Lung 7 mature OS: Paz-Ares et al., Annals of Oncology 2017

Evidence for this treatment setting

The mutation and prior treatment matter.

Study groups cannot predict your individual outcome or compare treatments from different trials.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00221 v5 · official regimen · EMA Giotrif product information (updated 11 May 2026)

THE WHOLE TREATMENT JOURNEY

Afatinib monotherapy: treatment and checks

Your treatment pattern

Daily oral treatment without a rest week

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EGFR-mutated advanced NSCLC · EGFR-TKI-naïve

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Treatment NCCP 00221 · SMPC	Confirm indication and prescribed dose	Daily afatinib; no rest week	Continue the prescribed dose	Review benefit, harms and dose	Agree the next treatment or follow-up
Blood tests NCCP 00221 · SMPC	Blood count, kidney and liver profile	Monthly tests; extra checks if unwell	Continue monthly tests	Investigate abnormal results or dehydration	Follow unresolved abnormalities
Heart and eyes NCCP 00221 · SMPC	Heart-pumping test if indicated; eye history	Report eye symptoms promptly	Heart tests if clinically indicated	Cardiac/ ophthalmology review if symptoms	Agree follow-up for unresolved problems
Symptoms and medicines NCCP 00221 · SMPC	Check symptoms, medicines and contact plan	Report early bowel/skin symptoms	Review adherence, supply and interactions	Confirm recovery before any restart	Continue advice for unresolved symptoms

Care / check	Before the first dose	Early treatment review	Ongoing daily treatment	Benefit / toxicity review	Treatment change or completion
Cancer assessment NCCP 00221 · SMPC	Confirm mutation, stage and previous treatment	Clinical review	Scans per Consultant plan	Discuss response or progression	Agree follow-up or next treatment

What each check is for

1



Confirm eligibility and the prescription

Before the first dose

The mutation and treatment setting determine applicability.

Ask / **act:** Document exact indication, dose and initial investigations.

2



Early bowel and skin review

From the first dose and early reviews

Early support can prevent dehydration and uncontrolled skin toxicity.

Ask / **act:** Record grade, duration, supportive treatment and recovery; apply medicine-specific thresholds.

3

New respiratory symptoms

At any time

Possible interstitial lung disease (ILD) requires urgent investigation.

Ask / **act:** Arrange urgent assessment and an authorised medicine plan.

4



Benefit and next-plan review

At planned cancer assessments or significant toxicity

Continuous treatment is not an unlimited automatic prescription.

Ask / **act:** Agree continuation, hold, restart, change or follow-up.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Tests and symptoms guide safe treatment decisions.

Before starting

Confirm the activating EGFR test, previous treatments and prescription. Check blood count, kidneys and liver. A heart-pumping test (echocardiogram (heart ultrasound) (ECHO) or multigated acquisition (heart pumping) scan (MUGA)) is used if clinically indicated. Review lungs, bowel, eyes, skin, all medicines and pregnancy precautions.

During treatment

NCCP specifies blood count, kidney and liver tests monthly. Heart-function tests are repeated if clinically indicated. Report diarrhoea and skin changes early, especially in the first weeks. Scans and cancer reviews follow your Consultant's plan.

Taking afatinib

Follow your individual prescription.

Daily tablets

Take at the same time daily without food: leave at least 3 hours after food before the dose and wait at least 1 hour afterwards before eating. Swallow with water. If missed, take it the same day unless the next dose is due within 8 hours; then skip. Never double a dose or repeat a vomited dose. Ask pharmacy for the approved water-dispersion method if swallowing is difficult.

Dose changes

Afatinib dose changes are usually in 10 mg steps. Only the team may consider 50 mg after the initial 3 weeks if conditions are met; previous reductions rule out escalation. If 20 mg cannot be tolerated, a different treatment plan may be needed.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily tablets, checks and decisions

No dates or personal information are needed. Confirm your own appointments with the team.

Treatment or check	Timing in this view
Preparation	**Before the first dose** Confirm the activating EGFR test, indication, prescription and baseline tests.
Daily oral treatment	**Early treatment review → Ongoing daily treatment → Benefit / toxicity review** The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.
Tests and symptoms	**Early treatment review → Ongoing daily treatment → Benefit / toxicity review → Treatment change or completion** NCCP specifies blood count, kidney and liver tests monthly. Heart-function tests are repeated if clinically indicated. Report diarrhoea and skin changes early, especially in the first weeks. Scans and cancer reviews follow your Consultant's plan.
Next treatment plan	**Treatment change or completion** Review response, side effects and whether to continue, change or stop treatment.

Take at the same time daily without food: leave at least 3 hours after food before the dose and wait at least 1 hour afterwards before eating. Swallow with water. If missed, take it the same day unless the next dose is due within 8 hours; then skip. Never double a dose or repeat a vomited dose. Ask pharmacy for the approved water-dispersion method if swallowing is difficult.

Relative stages, not equal lengths of time or booked appointments. Daily tablets continue between reviews unless the team tells you to hold. Confirm who arranges and reviews every test.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Living with treatment

Report symptoms early and keep contact details available.

Diarrhoea and fluids

Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.

Skin, nails and mouth

Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.



Nausea medicines

Afatinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.

NCCP 00221 v5 · official regimen · NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 (2025), oral tables · NCCP NCIS antiemetic medicines V8 (2025); parenteral scope



Medicine interactions

Give pharmacy a list of all medicines, herbal products and supplements. Some P-gp inhibitors need separated dosing; strong inducers such as St John’s wort can reduce afatinib exposure. Ask for a written individual schedule before adding or changing medicines.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 1 month after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

When to get urgent help

Do not wait for the next appointment.

Call urgently

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

When you call

Give the medicine name, last dose and symptoms. Tell the team if you can drink, pass urine and manage usual activities. Do not delay calling while gathering information.

When treatment changes

The team authorises continuation, interruption, restart or stopping.

Before restarting

Confirm symptom recovery, blood results, the prescribed dose and the next review.

When treatment ends

Agree follow-up, unresolved symptom care and reproductive precautions.

Practical support

Diarrhoea and fluids

Skin, nails and mouth

Nausea medicines

Medicine interactions

Pregnancy, fertility and breastfeeding

Taking tablets / attending treatment

Practical treatment details

Supplies and visits

Arrange the next prescription before tablets run out.

The oral regimen itself does not require an infusion line or pump.

Questions to ask

Is there a planned week off?

No. These tablets are continuous daily treatment. A hold for side effects is a clinical decision.

Can I adjust the dose myself?

No. Keep the prescription and contact plan. Your team authorises every hold, restart or dose change.

Do I need all the tests every day?

No. Your team schedules tests and reviews. Contact them between appointments if symptoms occur.

Explain the plan back: Can you explain how to take your tablets, what to do after a missed or vomited dose, which tests are next and which symptoms mean you should call today?

Which treatment this guide covers NCCP 00221a covers adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Afatinib alone · daily oral treatment

Active medicines: afatinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.
- Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.
- Afatinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.
- Give pharmacy a list of all medicines, herbal products and supplements. Some P-gp inhibitors need separated dosing; strong inducers such as St John’s wort can reduce afatinib exposure. Ask for a written individual schedule before adding or changing medicines.
- Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 1 month after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Which treatment this guide covers National Cancer Control Programme (NCCP) 00221a covers adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

NCCP 00221 · VERSION 5

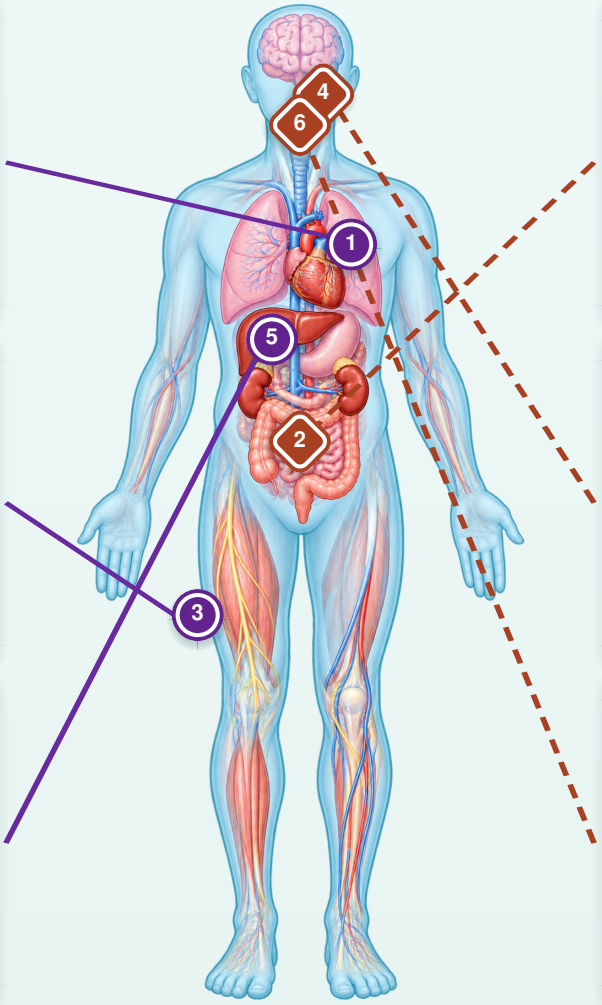
Afatinib monotherapy

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

afatinib

Possible lung inflammation needs urgent assessment.

2 Diarrhoea or severe abdominal pain

afatinib

Dehydration can injure the kidneys; severe abdominal pain may need emergency assessment.

3 Rash, nail changes or blistering

afatinib

Report early; severe blistering or peeling is urgent.

4 Eye pain or blurred vision

afatinib

Possible keratitis needs prompt specialist review.

5 Yellow eyes or skin

afatinib

Liver injury may need blood tests and treatment review.

6 Mouth soreness

afatinib

Support can help eating, drinking and mouth care.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00221 v5 · official regimen**
   https://healthservice.hse.ie/documents/6936/221_Afatinib.pdf
2. **EMA Giotrif product information (updated 11 May 2026)**
   https://www.ema.europa.eu/en/documents/product-information/giotrif-epar-product-information_en.pdf
3. **NCCP V6 oral minimal-low: individual as-required antiemetics; NCIS V8 is parenteral and does not prescribe an oral course**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS antiemetic medicines V8 (2025); parenteral scope**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **LUX-Lung 3 primary PFS: Sequist et al., JCO 2013**
   https://pubmed.ncbi.nlm.nih.gov/23816960/
6. **LUX-Lung 3/6 mature OS: Yang et al., Lancet Oncology 2015**
   https://pubmed.ncbi.nlm.nih.gov/25589191/
7. **LUX-Lung 7 mature OS: Paz-Ares et al., Annals of Oncology 2017**
   https://pubmed.ncbi.nlm.nih.gov/28426106/